Version:1

Bwrdd Iechyd Prifysgol
Betsi Cadwaladr
University Health Board

ANAESTHESIA, CRITICAL CARE & PAIN MANAGEMENT DIRECTORATE

OBSTETRIC ANAESTHESIA WRITTEN CONTROL DOCUMENT

MANAGEMENT OF ECLAMPSIA

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1. Introduction and Purpose

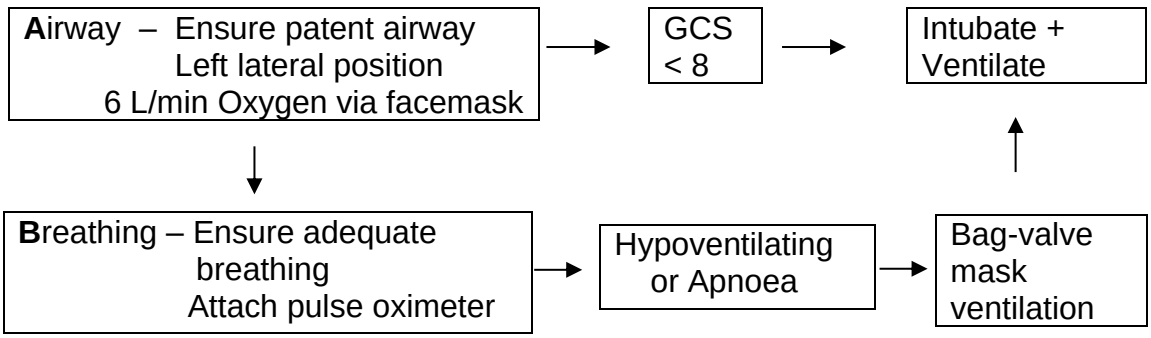
Eclampsia is defined as the development of convulsions or coma in women with pre-eclampsia, without other precipitating causes of seizures. It occurs as a progression of pre-eclampsia with the convulsions being the result of cerebral ischaemia. Management of these patients should be prompt to avoid maternal and fetal morbidity and mortality, and is the joint responsibility of the obstetrician and anaesthetist.

2. Scope

This guideline is intended as a reference for trainee anaesthetists seeking advice regarding the management of eclampsia.

3. Points to consider

ABC Management



Circulation –

- **Check that aortocaval occlusion is corrected** (15 degree left lateral tilt if patient is supine)
- **Assess pulse** (volume and rate) **and BP**
- **Attach continuous ECG**
- **Insert a 14G (or 16G) iv cannulae** for intravenous access
  Take blood for X – matching, FBC, clotting screen & serum U+E's.
- Control fits by giving a **loading dose of 4g of MgSO4 IV** over 5 minutes
- **Commence iv infusion** of crystalloid or colloid
- Prevent further fits by commencing an **infusion of MgSO4 at 1g/hr** for at least 24 hours after last seziure
- Subsequent fits should be controlled with an additional 2g bolus of MgSO4 over 5 minutes.
- If fitting is not controlled with MgSO4, or the patient has a GCS < 8 for a prolonged period of time, then induce anaesthesia and intubate using a rapid sequence technique (with thiopentone). Ventilate the patient using a bag valve mask and transfer to theatre, keeping the patient paralysed and sedated with rocuronium and midazolam.
- Once fits are controlled, if patient is antenatal then await decision of obstetrician as to plan for delivery.If a Caesarean Section is needed, then give a general anaesthetic
- Ensure **continuous CTG** monitoring of fetus
- **Insert an urinary catheter** - To measure hourly urine output
- **Insert an arterial line** - To monitor invasive arterial Bp and take further blood samples including ABG's
- **Consider inserting a CVP line** - To guide further fluid therapy. Insert after initial ABC measures have been completed
- **Transfer to ITU or OHOU** - Following initial stabilisation, with or without surgical intervention transfer to ITU if the patient needs to remain intubated and ventilated. Otherwise transfer to Obstetric High Observation Unit (OHOU) for further care and monitoring of vital parameters (Pulse, BP, CVP, temperature, oxygen saturation, respiratory rate, urine output and conscious level)

4. References

1. The Eclampsia Trial Collaborative Group. Which anticonvulsant for women with eclampsia? Evidence from the collaborative eclampsia trial. *Lancet* 1995; 345:1455 – 1463

2. Yentis SM. The Magpie has landed: pre-eclampsia, magnesium sulphate and rational decisions. *International Journal of Obstetric Anaesthesia* 2002; 11: 238 -241

3. Anaesthesia and pre-eclampsia / eclampsia. Norris M.C. (2000) *Handbook of Obstetric Anaesthesia.* First Edition. Lippincott Williams & Wilkins : 25 – 103

4. Management of Eclampsia. *PROMPT Course Manual* Edition 3 (2017) Cambridge University Press Page 102 - 107